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June 16, 2025

Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
Department of Health and Human Services
Mary E. Switzer Building, 7033A
330 C Street SW
Washington, DC 20201
Attention: Centers for Medicare & Medicaid Services [CMS–0042–NC] RIN 0938–AV68 Request for Information; Health Information; Health Technology Ecosystem

Dear Assistant Secretary for Technology Policy (ASTP)/ Office of the National Coordinator for Health Information Technology (ONC), and Department of Health and Human Services (HHS):

As an integral part of Colorado's healthcare and innovation ecosystem, Colorado Community Managed Care Network/Community Health Provider Alliance (CCMCN/CHPA) appreciates the opportunity to submit comments on the Office of the Assistant Secretary for Technology Policy and Office of the National Coordinator for Health Information Technology (ASTP/ONC) and Department of Health and Human Services (HHS), Request for Information (RFI), Heath Technology Ecosystem, CMS–0042–NC. We are submitting our comments in collaboration with two of our partner organizations in the Colorado health technology and health care access space, Colorado Access, the state's largest public sector health plan, and Prime Health, a hub for accelerating health innovation and convening cross-sector partners. We each focus our comments on the health programs and systems questions posed by the RFI that fall in our intersecting or particular expertise and organizational experience. As preferred, our responses are grouped by topic, headed with the applicable question numbers from the RFI.

If you have any questions or would like further information, please contact Jason Greer, CEO, CCMCN/CHPA, Ashlie Brown, Chief Strategy & Technology Officer, Colorado Access, or Sarah Lampe, President & CEO, Prime Health

Thank you again for inviting our input,

Jason Greer
CEO, CCMCN/CHPA

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Colorado Community Managed Care Network (CCMCN)

DBA Community Health Provider Alliance (CHPA)

CMS Health Technology Ecosystem RFI Response

Organization: Colorado Community Managed Care Network (CCMCN) DBA Community Health Provider Alliance (CHPA)
Website: CCMCN.org and CHPANetwork.com
Contact: Jason Greer, CEO, CCMCN/CHPA, jason@ccmcn.org

Coordination Note: Colorado is uniquely positioned to pilot transformative health policy innovation. This response is part of a coordinated submission representing Prime Health Colorado, Colorado Access, and Colorado Community Managed Care/Community Health Provider Alliance, each a key ecosystem player with deep statewide partnerships across government agencies, provider organizations, philanthropies, research institutions, and startups.

Colorado serves as a natural living laboratory with proven experience building collaborative health ecosystems. Unlike coastal markets, Colorado's geography, encompassing urban, suburban, rural, and frontier regions, mirrors the national landscape, making it ideal for scalable solutions.

Fragmented social health information regulations challenge Colorado and the nation. **Colorado's collaborative track record positions it to lead testing of breakthrough policy, funding and data solutions,** including mandating open architecture with enforcement funding, intentionally funding interoperable ecosystems over individual data monetization, and establishing innovation best practices and standards for Medicaid/Medicare populations. Each of our organizations sees challenges and opportunities around Policy, Data and Funding and how they interact, and have responded to RFI questions pertinent to each of our experience and expertise.

Organization Overview

CCMCN/CHPA is a unified care network focused on improving patient outcomes through coordinated care management, value-based care, quality improvement, and health technology integration. We work across Colorado's diverse healthcare landscape to connect providers, payers, and patients through innovative care coordination platforms and data sharing initiatives.

CCMCN started as a network of 19 Community Health Centers that was founded in 1994 to respond proactively to the advent of mandatory Medicaid managed care, and evolved to a

multi-faceted organization focusing on managed and accountable care, health information technology, and clinical quality improvement programming. CHPA is an accountable care organization focused on supporting performance in value-based care contracts. Merging together in 2025 to consolidate efforts in value-based care and integrated healthcare services, our new organization strengthens Colorado's community health network through value-based care innovation, data-driven solutions, and collaborative clinical support. Our expertise lies in supporting provider organizations with technology, quality improvement, and managed care solutions, including community data integration; CCMCN/CHPA and our member organizations have direct insights into how technology changes would impact frontline care delivery and patient outcomes.

Key RFI Responses

PR-7: Supporting High-Quality Data Availability

CCMCN/CHPA's Care Coordination Data Challenges:

- Fragmented EHR systems across 200+ provider partners
- Inconsistent data quality and completeness across care settings
- Limited real-time data sharing capabilities
- Complex care transitions requiring manual coordination
- Duplicate data entry burdens on both providers and patients
- Lack of standardized data exchange with non-healthcare entities (justice system, community-based organizations, other HHS programs)

Strategies for CMS Implementation:

- Standardize care coordination data elements across all provider types
- Incentivize real-time data sharing through quality bonus payments
- Establish interoperability requirements for care management platforms
- Create data quality metrics tied to provider network participation
- **Mandate data exchange capabilities with non-healthcare entities including justice systems, CBOs, and HHS programs**
- **Ensure claims data is readily available under HIPAA for care coordination purposes**
- **Reduce duplicate data entry through automated population of forms across systems**

Burden Mitigation Approaches:

- Automated data extraction from existing EHR workflows
- Shared care coordination platforms reducing duplicate data entry
- Integration with existing quality reporting requirements

- Technical assistance for smaller provider practices
- **AI-powered documentation and coding tools to decrease provider administrative burden**
- **CMS subsidization of health technology tools for both patients and providers**

VB-3: Essential Health IT Capabilities for Value-Based Care

Critical Capabilities from CCMCN/CHPA Network Experience:

1. **Care Team Coordination:**
    - Real-time communication platforms connecting primary care, specialists, behavioral health, and social services
    - Shared care plan development and tracking across multiple providers
    - Task assignment and completion tracking for care team members
    - **Community care coordination systems linking healthcare with other service agencies around individual patient needs**
2. **Population Health Management:**
    - Risk stratification tools identifying high-need, high-cost patients
    - Care gap identification and closure tracking
    - Social determinants screening, resource connection and social data sharing
    - Chronic disease management program integration
    - **AI-powered need assessment and resource routing capabilities**
    - **Community connection platforms to address social isolation and loneliness**
3. **Quality Measurement and Reporting:**
    - Automated quality metric calculation from EHR data
    - Real-time performance dashboards for providers and payers
    - Outcome tracking across care episodes and provider networks
    - Patient-reported outcome measurement integration
    - Quality measure alignment to ease reporting burden and provide actionable insights aligned across programs
4. **Patient-Centered Technology:**
    - **Individualized patient portals that don't require separate logins for each provider**
    - **Extremely user-friendly interfaces designed for non-technical users**
    - **AI assistants for first-line patient support and triage**
    - **Integrated scheduling services across multiple providers and agencies**
    - **Patient control over sharing of both health and social information**

5. **Benefits and Program Integration:**
    - **Enhanced SNAP/WIC/Medicaid integration for easier enrollment and eligibility verification**
    - **Cross-program enrollment facilitation even when administered by different organizations**
    - **Online tools to capture patient needs and route them to appropriate support services**

TD-6: TEFCA Unique Interoperability Functions

CCMCN/CHPA's TEFCA Implementation Planning: We see TEFCA as essential for our care coordination mission:

Unique Functions:

- Standardized patient matching across disparate health systems
- Secure care coordination messaging between unaffiliated providers
- Clinical data sharing for care transitions and referrals
- Quality measure data aggregation across multiple networks
- **Integration with social services and community organization data systems**
- **Cross-sector data sharing for comprehensive care coordination**

Current Limitations:

- Complex onboarding process for smaller provider practices
- Limited behavioral health provider participation
- Inconsistent implementation across Colorado health systems
- Cost barriers for rural and safety net providers
- **Lack of funding mechanisms for HIE participation and maintenance**

Recommended Changes:

- Simplified participation pathway for care coordination organizations
- Dedicated funding for rural provider TEFCA implementation
- Behavioral health-specific data sharing protocols
- Integration with existing state health information exchanges
- **Government funding for HIEs with provider payment for participation**
- **Mandatory participation requirements with adequate financial support**

PR-5: FHIR API Implementation Status

Current CCMCN/CHPA FHIR Capabilities:

- Patient Access API: Implemented across 60% of network providers
- Provider Directory API: Full implementation supporting care coordination
- Care Plan API: Pilot implementation for chronic disease management
- Quality Measure API: Development phase for value-based contracts

Implementation Challenges:

- Varying EHR vendor FHIR maturity levels
- Limited technical support in rural provider practices
- Complex authentication requirements for care team access
- Inconsistent data element availability across providers
- **Need for enhanced user-friendly interfaces for non-technical staff**
- **Integration challenges with social services and community organization systems**

Colorado Network-Specific Needs

Rural Care Coordination

- Telehealth platform integration with care coordination workflows
- Transportation coordination data sharing protocols
- Critical Access Hospital-specific interoperability requirements
- Weather and seasonal access considerations in care planning
- **Enhanced technology subsidies for rural providers and patients**

Behavioral Health Integration

- Mental health provider network interoperability
- Substance abuse treatment data sharing protocols
- Crisis intervention coordination systems
- Integrated primary care and behavioral health platforms
- **AI-powered mental health screening and triage tools**

Social Determinants Integration

- Connection with Colorado's 211 resource database
- Housing services coordination and tracking
- Food security screening and resource connection
- Transportation services integration and scheduling
- **Enhanced SNAP/WIC integration and automated eligibility screening**
- **Cross-agency benefit enrollment and management systems**

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Policy Recommendations

Care Coordination Platform Requirements

- Mandate interoperability standards for all care coordination platforms
- Establish minimum data sharing requirements for network participation
- Create quality metrics specific to care coordination effectiveness
- Incentivize real-time data sharing through payment model adjustments
- **Require cross-sector data sharing capabilities with non-healthcare entities**
- **Mandate patient-controlled data sharing mechanisms**

Network Participation Standards

- Develop tiered participation requirements accommodating different provider sizes
- Establish technical assistance programs for smaller practices
- Create shared services models for care coordination technology
- Support regional care coordination network development
- **Provide government subsidies for health technology tools**
- **Pay providers for HIE participation and data sharing**

Quality Measurement Integration

- Align care coordination metrics with existing quality reporting
- Establish outcome measures for care coordination effectiveness
- Create shared accountability metrics across provider networks
- Integrate patient experience measures for care coordination
- **Include community connection and social isolation metrics**

ACO Enhancement

- **Provide additional funding for HIT tools specifically for ACOs**
- **Support AI-powered care management and population health tools**
- **Enable cross-sector partnerships through enhanced data sharing capabilities**

Data Integration Priorities

Most Critical for Care Coordination:

- Real-time ADT feeds from all hospitals and emergency departments
- Behavioral health encounter data across the care continuum
- Social services utilization data from community organizations
- Transportation services usage and care access barriers

- Pharmacy adherence data for chronic disease management
- **Justice system interaction data for comprehensive care planning**
- **Benefits enrollment and utilization data across all programs**
- **Community-based organization service delivery data**

Patient-Centered Technology Vision

Essential Patient-Facing Capabilities:

- **Single, individualized patient portal accessible across all providers**
- **AI-powered virtual assistants for 24/7 support and triage**
- **Automated benefits screening and enrollment across all eligible programs**
- **Community resource connection and social networking features**
- **Simplified, non-technical user interfaces with multilingual support**
- **Patient-controlled data sharing with granular privacy controls**
- **Integrated scheduling across healthcare and social service providers**

Commitment to Advancement

CCMCN/CHPA commits to:

- Serving as pilot network for enhanced interoperability initiatives
- Sharing care coordination best practices and lessons learned
- Supporting evaluation of care coordination technology effectiveness
- Collaborating on development of rural-appropriate care coordination solutions
- **Testing patient-centered technology solutions with comprehensive feedback mechanisms**
- **Piloting cross-sector data sharing initiatives with community partners**
- **Demonstrating cost-effective models for government-subsidized health technology**

Our coordinated response with Colorado's health ecosystem partners demonstrates our shared commitment to advancing care coordination through improved health technology infrastructure, patient-centered design, and comprehensive policy support that addresses the full spectrum of individual and community needs.